

1. Name of the medicinal product

FENOTASH (Fenofibrate 200mg Capsules)

2. Qualitative and quantitative composition

Each capsule contains 200 mg fenofibrate.

For the full list of excipients, see section 6.1.

3. Pharmaceutical form

Hard gelatin capsule. (Gelatin of Bovine Source)

Size 1 hard gelatin capsules having yellow cap and clear transparent body filled with white to off-white spherical pellets, packed in clear transparent PVC-Aluminium blister

4. Clinical particulars

4.1 Therapeutic indications

Fenofibrate Capsules indicated as an adjunctive therapy to diet for treatment of endogenous hypertriglyceridemia isolated (type IV), hypercholesterolemia (type IIa) or combined hypertriglyceridemia (type IIb and III).

4.2 Posology and method of administration

Patients should be placed on an appropriate lipid-lowering diet before receiving Fenotash, and should continue this diet during treatment. The recommended dosage is one Fenotash capsule daily with the main meal

4.3 Contraindications

- Hepatic insufficiency (including biliary cirrhosis and unexplained persistent liver function abnormality),
- Known gallbladder disease,
- Severe renal insufficiency (estimated glomerular filtration rate $< 30 \text{ mL/min/1.73 m}^2$),
- Chronic or acute pancreatitis with the exception of acute pancreatitis due to severe hypertriglyceridemia,
- Known photoallergy or phototoxic reaction during treatment with fibrates or ketoprofen,
- Hypersensitivity to the active substance or to any of the excipients listed in section 6.1.

4.4 Special warnings and precautions for use

Secondary causes of hyperlipidemia:

Secondary causes of hyperlipidemia, such as uncontrolled type 2 diabetes mellitus, hypothyroidism, nephrotic syndrome, dysproteinemia, obstructive liver disease, pharmacological treatment, alcoholism, should be adequately treated before fenofibrate therapy is considered. Secondary cause of hypercholesterolemia related to pharmacological treatment can be seen with diuretics, β -blocking agents, estrogens, progestogens, combined oral contraceptives, immunosuppressive agents and protease inhibitors. In these cases it should be ascertained whether the hyperlipidaemia is of primary or secondary nature (possible elevation of lipid values caused by these therapeutic agents).

Liver function:

As with other lipid lowering agents, increases have been reported in transaminase levels in some patients. In the majority of cases these elevations were transient, minor and asymptomatic. It is recommended that transaminase levels are monitored every 3 months during the first 12 months of treatment and thereafter periodically. Attention should be paid to patients who develop increase in transaminase levels and therapy should be discontinued if

AST (SGOT) and ALT (SGPT) levels increase to more than 3 times the upper limit of the normal range. When symptoms indicative of hepatitis occur (e.g. jaundice, pruritus), and diagnosis is confirmed by laboratory testing, fenofibrate therapy should be discontinued.

Pancreas:

Pancreatitis has been reported in patients taking fenofibrate (see sections 4.3 and 4.8). This occurrence may represent a failure of efficacy in patients with severe hypertriglyceridaemia, a direct drug effect, or a secondary phenomenon mediated through biliary tract stone or sludge formation with obstruction of the common bile duct.

Muscle:

Muscle toxicity, including rare cases of rhabdomyolysis, with or without renal failure has been reported with administration of fibrates and other lipid-lowering agents. The incidence of this disorder increases in cases of hypoalbuminaemia and previous renal insufficiency. Patients with pre-disposing factors for myopathy and/or rhabdomyolysis, including age above 70 years, personal or familial history of hereditary muscular disorders, renal impairment, hypothyroidism and high alcohol intake, may be at an increased risk of developing rhabdomyolysis. For these patients, the putative benefits and risks of fenofibrate therapy should be carefully weighed up.

Muscle toxicity should be suspected in patients presenting diffuse myalgia, myositis, muscular cramps and weakness and/or marked increases in CPK (levels exceeding 5 times the normal range). In such cases treatment with fenofibrate should be stopped.

The risk of muscle toxicity may be increased if the drug is administered with another fibrate or an HMG-CoA reductase inhibitor, especially in cases of pre-existing muscular disease. Consequently, the co-prescription of fenofibrate with a HMG-CoA reductase inhibitor or another fibrate should be reserved to patients with severe combined dyslipidaemia and high cardiovascular risk without any history of muscular disease and a close monitoring of potential muscle toxicity.

Renal function:

Fenofibrate Capsules is contraindicated in severe renal impairment (see section 4.3).

Fenofibrate Capsules should be used with caution in patients with mild to moderate renal insufficiency. Dose should be adjusted in patients whose estimated glomerular filtration rate is 30 to 59 mL/min/1.73 m² (see section 4.2).

Reversible elevations in serum creatinine have been reported in patients receiving fenofibrate monotherapy or co-administered with statins. Elevations in serum creatinine were generally stable over time with no evidence for continued increases in serum creatinine with long term therapy and tended to return to baseline following discontinuation of treatment.

Treatment should be interrupted when creatinine level is 50% above the upper limit of normal. It is recommended that creatinine is measured during the first 3 months after initiation of treatment and periodically thereafter.

4.5 Interaction with other medicinal products and other forms of interaction

Oral Anti-coagulants

Fenofibrate enhances oral anti-coagulant effect and may increase risk of bleeding. In patients receiving oral anti-coagulant therapy, the dose of anti-coagulant should be reduced by about one-third at the commencement of treatment and then gradually adjusted if necessary according to INR (International Normalised Ratio) monitoring.

Cyclosporin

Some severe cases of reversible renal function impairment have been reported during concomitant administration of fenofibrate and cyclosporin. The renal function of these patients must therefore be closely monitored and the treatment with fenofibrate stopped in the case of severe alteration of laboratory parameters.

HMG-CoA reductase inhibitors or Other Fibrates

The risk of serious muscle toxicity is increased if a fibrate is used concomitantly with HMG-CoA reductase inhibitors or other fibrates. Such combination therapy should be used with caution and patients monitored closely for signs of muscle toxicity (see section 4.4.).

There is currently no evidence to suggest that fenofibrate affects the pharmacokinetics of simvastatin.

Concurrent use of fibrates with HMG-CoA reductase inhibitors may cause severe myositis and myoglobinuria.

Glitazones

Some cases of reversible paradoxical reduction of HDL-cholesterol have been reported during concomitant administration of fenofibrate and glitazones. Therefore it is recommended to monitor HDL-cholesterol if one of these components is added to the other and stopping of either therapy if HDL-cholesterol is too low.

Cytochrome P450 enzymes

Data of human liver microsomes indicate that fenofibrate and fenofibric acid are not inhibitors of cytochrome (CYP) P450 isoforms CYP3A4, CYP2D6, CYP2E1, or CYP1A2. They are weak inhibitors of CYP2C19 and CYP2A6, and mild-to-moderate of CYP2C9 at therapeutic concentrations.

Patients co-administered fenofibrate and CYP2C19, CYP2A6, and especially CYP2C9 metabolised drugs with a narrow therapeutic index should be carefully monitored and, if necessary, dose adjustment of these drugs is recommended.

Other

In common with other fibrates, fenofibrate induces microsomal mixed-function oxidases involved in fatty acid metabolism in rodents and may interact with drugs metabolised by these enzymes.

4.6 Fertility, pregnancy and lactation

Pregnancy:

There are no adequate data from the use of fenofibrate in pregnant women. Animal data have not demonstrated any teratogenic effects. Embryotoxic effects have been shown at doses in the range of maternal toxicity. The potential risk for humans is unknown. Therefore, Fenofibrate Capsules should only be used during pregnancy after a careful benefit/risk assessment.

Lactation:

It is unknown whether fenofibrate and/or its metabolites are excreted in human milk. A risk to the suckling child cannot be excluded. Therefore fenofibrate should not be used during breast-feeding.

Fertility:

Reversible effects on fertility have been observed in animals. There are no clinical data on fertility from the use of Fenofibrate Capsules.

4.7 Effects on ability to drive and use machines

Fenofibrate Capsules has no or negligible influence on the ability to drive and use machines.

4.8 Undesirable effects

The most commonly reported ADRs during Fenotash therapy are digestive, gastric or intestinal disorders. The following undesirable effects have been observed with the below indicated frequencies:

MedDRA system organ class	Common	Uncommon	Rare	Very rare
Blood and lymphatic system disorders			Haemoglobin decreased White blood cell count decreased	
Immune system disorders			Hypersensitivity	
Nervous system disorders		Headache		
Vascular disorders		Thromboembolism (pulmonary embolism, deep vein thrombosis)		
Gastrointestinal disorders	Gastrointestinal signs and symptoms (abdominal pain, nausea, vomiting, diarrhoea, flatulence)	Pancreatitis		
Hepatobiliary disorders	Transaminases increased (see section 4.4)	Cholelithiasis (see section 4.4)	Hepatitis	
Skin and subcutaneous tissue disorders		Cutaneous hypersensitivity (e.g. Rashes, pruritus, urticaria)	Alopecia Photosensitivity reactions	
Musculoskeletal, connective tissue and bone disorders		Muscle disorder (e.g. myalgia, myositis, muscular spasms and weakness)		
Reproductive system and breast disorders		Sexual dysfunction		
Investigations	Blood homocysteine level increased**	Blood creatinine increased	Blood urea increased	

** The average increase in blood homocysteine level in patients treated with fenofibrate was 6.5 µmol/L, and was reversible on discontinuation of fenofibrate treatment. The increased risk of venous thrombotic events may be related to the increased homocysteine level. The clinical significance of this is not clear.

In addition to those events, the following side effects have been reported spontaneously during postmarketing. A precise frequency cannot be estimated from the available data and is therefore classified as “not known”.

- Respiratory, thoracic and mediastinal disorders: Interstitial lung disease.
- Musculoskeletal, connective tissue and bone disorders: Rhabdomyolysis.
- Hepatobiliary disorders: jaundice, complications of cholelithiasis (e.g. cholecystitis, cholangitis, biliary colic)
- Skin and Subcutaneous Tissue Disorders: severe cutaneous reactions (e.g erythema multiforme, Stevens-Johnson syndrome, toxic epidermal necrolysis)
- General disorders and administration site conditions: Fatigue

4.9 Overdose

Only anecdotal cases of fenofibrate overdosage have been received. In the majority of cases no overdose symptoms were reported.

No specific antidote is known. If overdose is suspected, treat symptomatically and institute appropriate supportive measures as required. Fenofibrate cannot be eliminated by haemodialysis.

5. Pharmacological properties

5.1 Pharmacodynamic properties

Serum Lipid Reducing Agents/Cholesterol and Triglyceride Reducers/Fibrates. ATC code:C10 AB 05.

Fenofibrate is a fibric acid derivative whose lipid modifying effects reported in humans are mediated via activation of Peroxisome Proliferator Activated Receptor type α (PPAR α). Through activation of PPAR α , fenofibrate increases lipolysis and elimination of atherogenic triglyceride rich particles from plasma by activating lipoprotein lipase and reducing production of Apoprotein C-III. Activation of PPAR α also induces an increase in the synthesis of Apoproteins A-I, and A-II.

There is evidence that treatment with fibrates may reduce coronary heart disease events but they have not been shown to decrease all cause mortality in the primary or secondary prevention of cardiovascular disease.

Data on use of fenofibrate on lipoprotein fractions show decreases in levels of LDL and VLDL cholesterol. HDL cholesterol levels are frequently increased. LDL and VLDL triglycerides are reduced. The overall effect is a decrease in the ratio of low and very low density lipoproteins to high density lipoproteins. Apolipoprotein-A and apolipoprotein-B levels are altered in parallel with HDL and LDL and VLDL levels respectively.

Extravascular deposits of cholesterol (tendinous and tuberous xanthoma) may be markedly reduced or even entirely eliminated during fenofibrate therapy.

Plasma uric acid levels are increased in approximately 20% of hyperlipidaemic patients, particularly in those with type IV disease.

Patients with raised levels of fibrinogen treated with fenofibrate have shown significant reductions in this parameter, as have those with raised levels of Lp(a). Other inflammatory markers such as C Reactive Protein are reduced with fenofibrate treatment.

The uricosuric effect of fenofibrate leading to reduction in uric acid levels of approximately 25% should be of additional benefit in those dyslipidaemic patients with hyperuricaemia.

Fenofibrate has been shown to possess an anti-aggregatory effect on platelets in animals and in a clinical study, which showed a reduction in platelet aggregation induced by ADP, arachidonic acid and epinephrine.

5.2 Pharmacokinetic properties

Absorption:

Maximum plasma concentrations (C_{max}) occur within 4 to 5 hours after oral administration. Plasma concentrations are stable during continuous treatment in any given individual. The absorption of fenofibrate is increased when administered with food.

Distribution:

Fenofibric acid is strongly bound to plasma albumin (more than 99%).

Metabolism and excretion:

After oral administration, fenofibrate is rapidly hydrolysed by esterases to the active metabolite fenofibric acid.

No unchanged fenofibrate can be detected in the plasma. Fenofibrate is not a substrate for CYP 3A4. No hepatic microsomal metabolism is involved.

The drug is excreted mainly in the urine. Practically all the drug is eliminated within 6 days. Fenofibrate is mainly excreted in the form of fenofibric acid and its glucuronoconjugate.

In elderly patients, the fenofibric acid apparent total plasma clearance is not modified.

Kinetic studies following the administration of a single dose and continuous treatment have demonstrated that the drug does not accumulate.

Fenofibric acid is not eliminated during haemodialysis.

The plasma elimination half-life of fenofibric acid is approximately 20 hours.

6. Pharmaceutical particulars

6.1 List of excipients

Excipients: Sugar spheres, Hypromellose, Sodium Lauril sulfate, Dimethicone Emulsion 35%, Simethicone Emulsion 30%, Talc

Composition of the capsule shell: Gelatin, purified water, Sunset Yellow FCF, Quinoline Yellow, Titanium Dioxide

6.2 Incompatibilities

No effect noted to date.

6.3 Shelf life

36 months

6.4 Special precautions for storage

Store below 30°C.

6.5 Nature and contents of container

Pack of 10 capsules in blisters (PVC/Aluminium).

Such 10 blisters are packed into an outer carton with a pack insert.

6.6 Special precautions for disposal and other handling

Any unused medicinal product or waste material should be disposed of in accordance with local requirements.

7. Product Owner

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8. Manufacturer

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10. Date of revision of the text

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